

Barcode No **87943811**
Patient Name **Mr.SAJAD**
Age/Sex **22 YRS/Male**
Referred By **DR. ADEEB SINGH**

Lab No **16952501040002**
Reg Date **04/Jan/2025 06:30PM**
Sample Coll. Date **04/Jan/2025 04:55 PM**
Sample Rec.Date **05/Jan/2025 10:52 AM**

IMMUNOLOGY

Test Name With Methodology	Result	Unit	Biological Ref.Interval
Chlamydia Trachomatis IgG Antibodies			
Chlamydia Trachomatis IgG Serum ,ELISA	0.54	AI	<0.9 : Negative 0.9-1.0 : Grey zone >1.0 : Positive

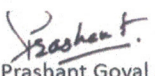
Medical Remarks: Kindly correlate clinically.

Interpretation:

- 1.Chlamydia Trachomatis IgG is intended for use in evaluating a patient's serologic status to Chlamydia Trachomatis infection. It is also used to evaluate paired sera for the presence of a significant increase in specific IgG as indicative of a recent or current Chlamydia Trachomatis infection.
- 2.High levels of anti-Chlamydia IgG antibody are of diagnostic value in chronic or systemic infections such as salpingitis, infertility, perihepatitis, epididymitis, Reiter's syndrome and pneumonitis.
- 3.It is one of most common sexually transmitted diseases. It is major cause of genital and urinary infection in men (nongonococcal urethritis, epididymitis). It also causes pelvic inflammatory disease including cervix, uterus, fallopian tubes and ovaries in females
- 4.During pregnancy this test provides an explanation for associated clinical signs and as well as complications. This prevents vertical transmission of maternal infection to fetus which may be harmed (inclusion conjunctivitis, pneumonia and other complications)

Limitations:

- 1.Cross reactivity with other Chlamydia species is known
- 2.In defining the immunity level of patient, the presence of IgG class antibodies at any level does not exclude the possibility of ongoing infection. Therefore specific IgA antibody testing is essential for an early diagnosis of acute infections
- 3.All results must be carefully interpreted and confirmed by clinical evaluation and further diagnostic tests like PCR.


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(Director & Chief Pathologist)
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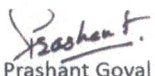
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Referred By	DR. ADEEB SINGH	Sample Rec.Date	05/Jan/2025 10:52 AM
Client	AP081899 Hexamed Healthcare Services LLP		
Code/Name		Report Date	05/Jan/2025 01:17PM
Ref. Lab/Hosp			

Test Name With Methodology	Result	Unit	Biological Ref.Interval
IMMUNOASSAY			
HSV 1&2 IGG,IGM (CLIA)			
HSV 1&2 IgG CLIA	34.30	AU/ML	<2.0 Negative.
HSV 1&2 IgM CLIA	0.03	AU/ML	<2.0 Negative 2.0-4.0 Gray Zone >4.0 Positive.

Comments :

This test is used to estimate the levels of I&II IgM antibodies against Herpes simplex virus (HSV) Herpes simplex virus (HSV) infection can cause "cold sores" affecting the lips and genital herpes. Both of these infections can recur. HSV is most commonly acquired through oral or genital contact. Newborns that contract the virus usually do so during travel through the birth canal of a woman who has a genital infection with HSV. The virus may spread throughout the newborn's body, attacking vital organs. Treatment with specific antiviral medication should begin as soon as possible in the infected newborn. Even if treated, surviving babies may have permanent damage to the central nervous system. The presence of HSV-1 or HSV-2 IgM antibodies indicates an active or recent infection. A negative test result indicates that the herpes simplex virus was not isolated but does not definitely rule out the presence of virus. Negative HSV antibody results mean that it is unlikely that the person has been exposed to HSV or that the body has not had time to begin producing HSV antibodies.


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Ref. Lab/Hosp		Report Date	05/Jan/2025 07:24PM

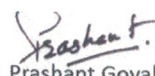
Test Name With Methodology	Result	Unit	Biological Ref.Interval
IMMUNOASSAY_SPE			
TPHA with titer (Treponema Pallidum Hemagglutination)			
TPHA..	NON REACTIVE		Non Reactive
<u>Dilution</u>			

COMMENTS:

Treponema pallidum hemagglutination assay (TPHA) is an indirect hemagglutination assay used for the detection of antibodies against the causative agent of syphilis, Treponema pallidum subspecies pallidum. Positive results indicate both past or present infections. False positive results are seen in patients suffering from Leprosy, Infectious mononucleosis and Connective tissue disorders. This test does not distinguish between Syphilis and other pathogenic treponemal infections.

Note: <1:80 Non Reactive

METHOD: Hemagglutination


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